



POLICY

Medication: Transdermal Patch Management

Scope (Staff):	North Metropolitan Health Service Mental Health Staff: Clinical Staff
Scope (Area):	North Metropolitan Health Service Mental Health: Inpatient Sites

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1. Aim

To ensure the safe use, monitoring and disposal of transdermal medication patches.

2. Background

There are a number of medications available in the transdermal patch (TDP) formulation and these may release the medication at different rates (See Appendix 1).

As many TDPs require intermittent or daily changing, there is a possibility of the TDP falling off unnoticed or being misplaced.

Minimum daily confirmation of TDP placement is considered to be best practice when the patch is left in situ for more than 24 hours. Daily checks by nursing staff for any evidence of local reaction are also recommended.

3. Risk

The risk of improper use or monitoring could result in under or over-dosing which may cause harm to patients.

4. Principles

4.1 General Considerations

- 1. There must be confirmation of the presence of a TDP on admission by the admitting nurse.
- 2. TDPs **must not** be cut as a means of reducing the dose.
- 3. Apply to clean, dry, hairless and intact skin.
- 4. Use a different site for each application, avoid same site for several days.
- 5. Wash hands with soap and water; apply gloves before applying the patch. Wash hands after applying the patch.

4.2 Transdermal Patch Check Label

The TDP check label was developed to prompt nursing staff to check at each shift that the:

- prescribed TDP is securely intact on the patient’s body,
- correct medicated TDP is in situ,
- correct strength TDP is in situ.

It is recommended that when a TDP is prescribed the TDP check label should be utilised.

Refer to the [Guidelines for the WA Hospital Medication Chart \(WA HMC\)](#) for information on how to use the TDP check label on the medication chart.

Figure 1. TDP check label

PATCH CHECK (each shift)	AM										
	PM										
	NIGHT										

The label should not be used for patches that are applied for less than 24 hours. The need for a patch check in this situation may cause confusion (where application occurs during one shift and its removal occurs at another shift).

Figure 2. Placement of the TDP check label on the medication chart

Date 10/1	Medication (print generic name) Buprenorphine Patch		Tick if Slow Release															
Route top	Dose 5 microg/hr	Frequency and NOW enter times weekly (SUN)	0800	X	X	X	X	X	X	X	X	X	X	X	X	X	X	X
Indication Pain relief	Pharmacy <i>[Signature]</i>	PATCH CHECK (each shift)	location AM															
Prescriber's signature A Doctor	Print your name A Doctor		PM															
			NIGHT															

4.3 Monitoring

1. The nurse applying the TDP must document the location of patch on the medication chart.
2. Nursing staff must initial under the relevant date and time on the medication chart during their shift to confirm the correct TDP is present and securely intact. This must occur at a minimum of once daily.

Figure 3. Example of TDP checking and documenting on medication chart

Date 4/1	Medication (print generic name) Buprenorphine Patch		Tick if Slow Release															
Route top	Dose 5 microg/hr	Frequency and NOW enter times weekly (TUES)	0800	X	AN	X	X	X	X	X	X	X	X	X	X	X	X	X
Indication Pain relief	Pharmacy <i>[Signature]</i>	PATCH CHECK (each shift)	location (R) upper arm															
Prescriber's signature A Doctor	Print your name A Doctor		AM	X	AN	RN	RN	AN										
			PM	X	CN	CP	CN											
			NIGHT	X	RN	DM	CP											

3. Before applying a new patch ensure the **removal** of the previous patch.
4. If the medication order has been ceased the medication/ treating nurse must ensure the patch has been removed.
5. Any evidence of local reaction should be reported to the patient's treating doctor, included in the handover of the patient and documented in the patient's integrated progress notes.

4.4 Multiple Patches

If multiple patches are required to administer a dose, the full dose should be prescribed, as a SINGLE ORDER. The patch check should be done for the TOTAL dose.

Example: Buprenorphine 15mcg patch prescribed; when the 15mcg patch is unavailable, two patches are used as a single medication order. Clinical pharmacist would endorse a 10mcg and a 5mcg patch to be used.

YEAR 20.....		DATE AND MONTH			6/7	7/7	8/7	9/7	10/7	11/7	12/7	13/7	14/7	15/7				
DOCTORS MUST ENTER administration times																		
Date 6/7	Medication (print generic name) Buprenorphine Patch		Tick if Slow Release															
Route top	Dose 15 microg/hr	Frequency and NOW enter times weekly TUES	0800	X	AN	X	X	X	X	X	X	X	X	X	X	X	X	X
Indication Pain relief	Pharmacy <i>[Signature]</i>	PATCH CHECK (each shift)	location (R) upper arm															
Prescriber's signature A Doctor	Print your name A Doctor		AM	AN	AN	RN	RN	AN										
			PM	CN	CN	CP	CN											
			NIGHT	RN	RN	DM	CP											

5. Disposal

5.1 Disposal of regular TDPs

When changing a patch, wear gloves, remove the used patch, fold it over on itself, bringing the adhesive sides together and dispose of safely in the dedicated sharps container or waste bin, out of reach of children.

5.2 Disposal of TDPs containing controlled medicines

There is a risk for diversion for controlled medicine patches. Opioid patches retain significant amounts of the active drug after the duration of their use. Refer to the [NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Procedure](#).

1. The disposal process including the removal from a patient must be witnessed by two authorised staff members (refer to the NMHS procedure).
2. Use gloves to avoid inadvertent contact with the patch.
3. Fold patch onto itself so the adhesive sides stick together, cut into small pieces with scissors and place in a secure mounted sharps clinical waste or secure pharmaceutical waste bin by two authorised staff members.
4. Record in the controlled medicines register and countersigned, by two authorised staff members. Record as a new transaction, in the back pages of the current register.

6. Unsecured or Missing Patches

Where a TDP is not securely in place or cannot be located, a new patch must be applied once a thorough check of the patient, patient's bed and surroundings is conducted.

The prescribing team must be informed and the incident documented in the patient's integrated progress notes.

The order will need to be rewritten as the day of the next patch change may be different.

If the TDP is not located during a routine check a Clinical Incident Management System (CIMS) form should be completed.

For missing controlled medicine TDPs, these require both the CIMS form and the medication discrepancy/ loss form to be completed. See the [Schedule 8 and restricted Schedule 4 Medicine discrepancy/loss report form](#).

7. Ceasing a Patch

When ceasing a patch if the patch check label is used this must be ceased by the prescriber at the same time. If not ceased by the prescriber a nurse or pharmacist may cease the patch check label. Documentation that the patch has been removed and disposed of next to the ceased prescription is required.

Figure 4. Ceasing a TDP

Date 6/7	Medication (print generic name) Buprenorphine Patch		Time 0800	Task X AN X X X X X X X X	Pat removed 10/17 AN 10/18	Yes / No Yes / No
Route top	Dose Smoking how WEEKLY	Frequency and NOW enter times Tues	Location Right upper arm			
Indication Pain relief	Pharmacy A Doctor	PATCH CHECK (each shift)	AM	AN AN RN RN AN	COASTED	Continue on discharge? Discharge? Duration: days Qty:
Prescriber's signature A Doctor	Print your name A Doctor		PM	CN CN CP CN	Dose increase	
			NIGHT	RN RN DM CP	A Doctor	

8. Precaution – Application of External Heat Source

Advise patients and their caregivers to avoid exposing the application site and surrounding area to direct external heat sources, such as heating pads or electric blankets, heat or tanning lamps, saunas, hot tubs, and heated water beds while wearing the patch because an increase in absorption may occur.

There is a potential for temperature dependent increases in buprenorphine released from the patch, thereby increasing the risk of opioid reactions.

9. Ordering the Patch Check labels

Name	Description	Product Code	Vendor	Quantity
Patch Check Label	USER APPLIED;"PATCH CHECK (EACH SHIFT)" 92MM x 15MM; BLACK TEXT ON BLUE BACKGROUND	TDPC100	The Label Factory	100 per roll

10. Compliance and Evaluation

- Audit of controlled substances in accordance with audit calendar by authorised pharmacy staff and reported in accordance with the NMHS DPP Audit Schedule.
- Monitoring reports of TDP related incidents in Datix CIMS by NMHS MH Safety, Quality and Performance Unit.

Related internal policies, procedures and guidelines

[NMHS MH Medication: Management of High Risk Medication Policy](#)

References

[Guidelines for the WA Hospital Medication Chart \(WA HMC\)](#)

[NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#)

[NMHS Destruction, Discard and Disposal of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Procedure](#)

[WA Reporting of Schedule 4 Restricted and Schedule 8 Medicines Discrepancies Policy](#)

[WA High Risk Medication Policy](#)

[National Safety and Quality Health Service Standards V2](#)

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Date First Issued:	10/07/2013	Last Reviewed:	26/02/2024	Review Date:	26/02/2027
Approved by:	NMHS MH Drugs and Therapeutics Committee (DTC)			Date:	11/03/2025
Endorsed by:	MHPHDS Executive Director			Date:	10/07/2013
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 4: Medication Safety				
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety		☒ Std 8: Governance, leadership and management ☒ Std 10: Delivery of Care		
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Document Version Control			
Date	Version	Author	Notes
2013	V1	NMHS MH Chief Pharmacist	Endorsed by MHEG; Initial endorsement (Minutes 183/13)
Dec 2021	V2	NMHS MH A/Chief Pharmacist	New template; updated information on documenting, monitoring and destruction of TDPs
Feb 2025	V2.1	NMHS MH A/Chief Pharmacist	Minor changes and formatting. Removed brands from appendix 1, many generic brands and discontinued brands.

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative ISD#4080

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APPENDIX 1. Table of Transdermal Patch Preparations

Transdermal Patch	Indication	Comments	Time on skin (application)
Buprenorphine (Controlled medicine)	Narcotic analgesic (opioid)	Apply to upper outer arm, upper chest, upper back or side of chest (not to large, scarred areas). Avoid exposure to external heat source.	7 days (avoid same area of application for 3 weeks)
Estradiol	Estrogen replacement therapy	Apply lower trunk or buttocks (place on covered site). Avoid exposure to external heat source.	7 days
Estradiol	Estrogen replacement therapy	Apply to abdomen. Avoid exposure to direct sunlight & solariums.	Twice weekly (every 3-4 days)
Estradiol/ norethisterone	Hormone replacement therapy	Apply to buttocks or abdomen.	Twice weekly (every 3-4 days)
Fentanyl (Controlled medicine)	Narcotic analgesic (opioid)	Apply to flat surface of torso or upper arms. Avoid exposure to external heat sources.	72 hours (3 days)
Glyceryl Trinitrate	Anti-angina	Apply to clean, dry, hair free, healthy intact skin of upper arm or chest.	24 hours (daily) Patch-off period of 8 -12 hrs at night - recommended to overcome tolerance
Lidocaine (Lignocaine)/ prilocaine	Anaesthetic – topical	Apply 1 hour before procedure to intact skin.	
Lidocaine (Lignocaine)	Neuropathic pain with previous herpes zoster infection	Nominated skin (after healing shingles). Avoid exposure to external heat sources.	12 hours on 12 hours off
Nicotine	Nicotine Replacement Therapy (NRT)	Apply upper thigh, hip, under arm or chest.	Daily (24 hr patch can be removed at night)
Nicotine 16 hour	Nicotine Replacement Therapy (NRT)	Apply upper thigh, hip, under arm or chest.	16 hours - applied on waking & remove at bedtime.
Oxybutynin	Bladder function disorders	Dry, intact skin on abdomen, hip or buttock. Caution in a hot environment (may cause fever & heat stroke).	Twice weekly (every 3-4 days)
Rivastigmine	Alzheimer's disease	Avoid exposure to external heat sources; only 1 patch/time.	24 hours (daily)
Rotigotine	Parkinson's Disease; Restless legs syndrome	Apply to abdomen, thigh, hip, flank, shoulder or upper arm. Avoid exposure to external heat sources.	24 hours (daily)
Testosterone	Testosterone replacement therapy	Apply to back, abdomen, upper arm or thigh.	24 hours (daily)